# Carboplatin every 3 weeks with head and neck radiotherapy

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Carboplatin with radiotherapy

**Active medicines:** Carboplatin

**Emetogenic risk:** high

| Medicine | Dose / route | When |
|---|---|---|
| Aprepitant | 125 mg orally on Day 1; 80 mg orally on Days 2 and 3 | Once daily on each listed day<br>Check interactions; follow the prescribed plan. |
| Ondansetron | 16 mg orally on Day 1 | Before treatment<br>NCIS permits individual splitting to 8 mg twice daily or reduction to 8 mg once daily. Review QT risk, electrolytes and constipation. |
| Dexamethasone | 12 mg orally on Day 1; 8 mg orally on Days 2, 3 and 4 | Once daily on each listed day<br>Confirm timing and diabetes/sleep advice. This is the dedicated carboplatin row. |
| Metoclopramide · duration requires prescription | 10 mg orally three times daily on subsequent days in the NCIS row | The source row does not specify an end day or PRN qualifier<br>Prescriber/pharmacist must specify duration and suitability, including renal function and neurological contraindications. Do not continue indefinitely. |

### Additional phase-specific support

- This carboplatin regimen has high sickness risk. Take the prescribed prevention and rescue medicines on their written schedule. Call if nausea or vomiting prevents drinking.
- Tell the team early if swallowing, appetite, pain or taste changes reduce intake. A dietitian can agree practical food, drink or feeding support with you. Do not wait for substantial weight loss.
- Agree a mouth-care plan and tell your dentist about previous or current cancer treatment. Arrange oncology advice before dental procedures, particularly after head and neck radiation.
- Ask for speech and language therapy if swallowing or speech is difficult. Use the food textures, exercises and communication support advised for you; a general guide cannot establish safe swallowing.
- Bring prescribed, over-the-counter and herbal medicines and supplements for review. Discuss fertility preservation and breastfeeding before treatment. Carboplatin can impair fertility. Do not breastfeed during treatment. The linked Irish product information advises contraception for women during treatment and at least six months afterwards; men should avoid fathering a child during treatment and at least three months afterwards. Confirm the actual product and personal advice.
- Continue mouth, swallowing, dental, skin and nutrition care after radiation. Blood tests and hearing or nerve review follow any persisting problem; scans and examinations follow your individual oncology and radiation plan.
- NCIS Table 1 allows clinician-selected ondansetron splitting to 8 mg twice daily or reduction to 8 mg once daily according to the individual patient.
- Review ondansetron QT risk and other QT-prolonging medicines/electrolytes.
- Check dexamethasone interactions, diabetes, sleep and infection effects; aprepitant/steroid interaction is already reflected in the specified row.
- Review metoclopramide renal function, neurological contraindications, duration and extrapyramidal risk; do not automatically stack with olanzapine.
- AUC reduction below 4 requires an individual prophylaxis review; history of emesis and patient factors may justify continuing stronger protection.
- No universal granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibiotic, antiviral or acid-suppression prescription is stated by the carboplatin protocol. G-CSF requires individual febrile-neutropenia risk review.
- Routine hypersensitivity premedication is usually unnecessary, but antiemetic prophylaxis is still required. Prior allergy/desensitisation is a separate specialist plan.
- Use the dedicated carboplatin AUC ≥4 row. It does not include routine olanzapine.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00261 v7 · pNCCP 00261 v7 p4; NCIS antiemetics v8 Table 1 pp4–5 · NCCP NCIS antiemetic medicines v8 · dedicated carboplatin AUC ≥4 row, pages 4–5